RFC: [Short, Specific Title]

Computer Science	Value
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Status	Proposed
Decision Date	(fill when decided)
Deciders	(who agreed to the outcome)
Superseded By	(link, if applicable)

Problem and Context

Our project is a therapist scheduling and workload management application intended to help therapists manage their workload and reduce the risk of burnout. It combines scheduling with a workload evaluation survey, personalized preferences, an AI advisor, and administrator controls.

Therapists complete a survey to evaluate their workload and provide information for personalized recommendations. They can also rank their preferences for working with different patient groups, such as patients experiencing depression. The application incorporates these rankings into its workload calculations, allowing sessions to contribute different amounts to a therapist's total workload. The AI advisor uses the workload evaluation and scheduling information to suggest action plans, such as adjusting the distribution of sessions or scheduling additional breaks. When a therapist reaches the maximum workload, a pop-up warning blocks additional session assignments. Administrators can also manage therapists' sessions, making consistent workload calculations and scheduling restrictions important across both user roles.

One dilemma our team faces is whether to deliver these features through a standalone website or an integration with SimplePractice. A website would give our team control over the surveys, AI advisor, workload calculations, and administrator interface. Integration could place these features closer to therapists' existing scheduling workflows, but it would require confirmed access and permission to enforce scheduling restrictions. Changing platforms after adoption could require substantial redevelopment and data migration. If we do nothing, the proposed workload evaluations, action plans, and automatic scheduling limits would remain unavailable, leaving therapists and administrators to manage these responsibilities through their existing processes.

Options

Option A: Build a standalone website

Therapists would open our application in a browser, enter appointment blocks and preferences, and review recommendations. We would control its interface, scheduling rules, and release schedule. This would let us test the concept without depending on another company's approval.

Its costs include development, hosting, database services, backups, security maintenance, and user support. Therapists would also spend time learning another interface and potentially entering information twice.

Option B: Build a feature integrated into SimplePractice

Therapists could receive recommendations within or alongside their existing workflow. With suitable authorized access, integration could reduce repeated entry and provide more current scheduling information.

However, integration requires confirmed technical capabilities and permission. Costs could include integration development, partner coordination, access fees, testing, and ongoing compatibility work.

Option C: Do Nothing

Therapists and administrators would continue using their current scheduling system without our proposed workload management features. This avoids additional development, hosting, and maintenance costs.

However, an availability-based scheduler may show an open appointment slot even when a therapist has already reached a manageable workload. Counting sessions alone may also overlook differences in session demands and therapist preferences. Without workload evaluations, personalized action plans, and enforced workload limits, therapists and administrators must identify overload and adjust schedules themselves.

Analysis

Analysis

Option A: Standalone Website

A standalone website gives us control over the surveys, AI advisor, workload calculations, and administrator tools. We can update features independently and enforce workload limits through the backend for both therapists and administrators. AI recommendations can load separately so they do not delay scheduling checks. However, we must build and maintain authentication, hosting, data storage, and security controls. Duplicate entry could also produce outdated schedules and inaccurate workload calculations. Costs include infrastructure, AI usage, maintenance, and users' time.

Option B: SimplePractice Integration

Integration could reduce duplicate entry and provide recommendations within therapists' existing workflow. However, performance and maintenance would depend on the platform's availability, access limits, and interface changes. Besides, read-only access might support evaluations without allowing us to block additional appointments. Integration also introduces coordination costs, possible access fees, and security responsibilities for information sent to our backend or AI advisor.

Option C: Do Nothing

Retaining the existing workflow avoids new development, hosting, and maintenance costs. However, therapists and administrators must continue evaluating workload manually. An open appointment slot may still be available when a therapist has reached a manageable workload. This option introduces the least technical complexity but provides none of our proposed surveys, personalized action plans, or automatic workload restrictions. Its ongoing costs are staff effort and missed opportunities to improve workload management.

Recommendation and Verification Plan

I recommend building a standalone website because it would give our team control over the required features without depending on unconfirmed integration access. We could implement the workload survey, patient preferences, AI advisor, and administrator tools together. Most importantly, we could enforce maximum workload limits consistently when either a therapist or an administrator assigns a session. Although this approach requires additional infrastructure and maintenance, it would allow us to develop and evaluate the application independently. Separating workload calculations from the interface would also make future integration easier.

A few criterias our team can evaluate this decision:

Development feasibility: Build a small end-to-end prototype to confirm that independent authentication, hosting, and deployment are achievable.

Operating costs: Estimate hosting, database, backups, AI usage, and support costs at expected usage levels.

Constraints

Licensing: A standalone website would require us to select libraries, hosting services, and an AI provider. Their licenses and service terms must permit our intended use, and we must follow any attribution or distribution requirements. SimplePractice integration would additionally depend on its access permissions and contractual terms.

Data use: Surveys, patient preferences, and appointment information may contain sensitive information. We should collect only what the application needs and use synthetic data during initial development. Before sending information to an AI provider, we must confirm the partner's authorization and the provider's storage, retention, and data-use terms.

Intellectual property: Ownership of the code, workload calculation method, and project materials must be clarified under our partner's agreements.

Delegation and Validation

I used AI to organize the RFC, draft sections, compare platform tradeoffs, and suggest verification criteria. I provided the project requirements and my preference for a standalone website. I checked the draft against the project requirements. I also rejected verification criteria that tested individual features instead of platform viability and redirected them toward development feasibility and operating costs.

Decision

Proposed, until your team decides. Then accepted, rejected, or superseded, with the date and who chaired the decision. If superseded, link the RFC that replaces this one.